

WEIGHT LOSS

Discontinuation, Maintenance & Weight Regain

Planning the exit at intake, and handling the conversation nobody prepares for

DOCUMENT WL-010	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Every patient stops eventually — by choice, by cost, by supply, by side effect, or by reaching their goal. This protocol covers maintenance strategy, planned discontinuation, the regain conversation, and the exit documentation that protects both the patient and the practice.

THE REGAIN CONVERSATION BELONGS AT INTAKE

Weight regain after stopping GLP-1 therapy is common and substantial, and patients are frequently told this for the first time when they are already off the drug and regaining. That sequence produces complaints, refund demands and reviews. Have the conversation before the first prescription and document it.

2 Maintenance Strategy

Approach	Description	Considerations
Continue at the effective dose	Ongoing therapy at the dose that achieved the goal.	Simplest and best supported. Requires ongoing monitoring, cost and supply.
Reduce to a lower maintenance dose	Step down to the lowest dose that holds the result.	Reasonable trial. Monitor for regain and be prepared to step back up.
Extend the dosing interval	Same dose, longer interval between administrations.	Off-label in approach. Requires explicit prescriber decision and documentation.
Planned discontinuation	Stop therapy with a structured lifestyle plan in place.	Requires realistic counseling on regain and a defined re-engagement pathway.

Whichever is chosen, record the rationale, the monitoring plan, and the criteria that would prompt a change of approach.

3 Planned Discontinuation

- 1
- Establish the reason** — goal achieved, cost, side effects, pregnancy planning, supply, or patient preference. The reason shapes the plan.

- 2 **Decide taper versus stop.** There is no withdrawal syndrome, but a stepped reduction gives a clearer view of where appetite settles and makes regain easier to detect early.
- 3 **Reinforce the lifestyle foundation** before the last dose, not after: protein target, resistance training, and a realistic eating pattern per WL-007.
- 4 **Set the monitoring plan.** Follow-up weights at defined intervals after stopping, with a specific point at which regain is reassessed.
- 5 **Agree the re-engagement threshold** in advance — the amount of regain at which the patient returns for review rather than waiting until they are back at baseline.
- 6 **Handle pregnancy planning specifically**, using the discontinuation interval set by your medical director, and coordinate with the patient's obstetric care.
- 7 **Write to the primary care physician** with a summary, with consent.
- 8 **Issue a discharge summary** to the patient covering therapy received, results achieved, and the plan going forward.

4 The Regain Conversation

Deliver this at intake, repeat it at maintenance planning, and repeat it again at discontinuation. Consistency between what was said at the start and what is said at the end is what determines whether regain becomes a complaint.

- Obesity is a chronic, relapsing condition, and these medications treat it while they are being taken.
- Substantial regain after stopping is common and is a physiological response, not a personal failure and not a defect in the treatment.
- Maintaining lean mass through protein and resistance training improves the odds after stopping — which is why WL-007 matters most at the exit.
- Regain is not a reason for shame, and returning for review is expected rather than an admission of failure.
- Re-initiation is possible and follows the full titration ladder again, not a resumption at the previous dose.

DO NOT PROMISE PERMANENCE

Marketing or counseling that implies the result is permanent once achieved is not supportable and is the root of most weight-loss program disputes. Neither staff nor advertising material should suggest it. Review your marketing against this standard specifically.

5 Supply Interruption & Shortage

- ☐ A written contingency plan for supply interruption, known to clinical staff.
- ☐ Patients informed proactively rather than discovering it at a scheduled appointment.
- ☐ Prescriber decision on whether to hold, reduce, or switch agents — documented per patient rather than applied as a blanket instruction.
- ☐ Switching agents follows the cross-titration requirements in WL-002, including restarting the ladder.
- ☐ Patients explicitly warned against sourcing product independently, with the risks explained and the counseling recorded.

- ☐ Alternative sourcing evaluated only through the WL-006 vetting process — never expedited because of urgency.
- ☐ Interruption, duration and restart decisions documented for every affected patient.

RESTART AFTER A GAP

Tolerance is lost during an interruption. A patient resuming after a significant gap generally re-initiates at a lower dose and re-titrates. Resuming at the pre-interruption dose is a common cause of severe GI adverse effects.

6 Exit Documentation

- Reason for discontinuation, and whether it was planned or patient-initiated.
- Final dose, total duration of therapy, and total weight change from baseline.
- Body composition change where measured.
- The regain counseling delivered, and when it was first delivered.
- Maintenance plan agreed, including the re-engagement threshold.
- Post-discontinuation follow-up arranged.
- Discharge summary issued to the patient.
- Letter to the primary care physician, with consent.
- Any outstanding referrals, and confirmation they were made.

7 Quick Reference

QUICK REFERENCE CARD

Stopping therapy

1. **Regain counseling at intake** — not at the exit.
2. **Establish the reason** for stopping; it shapes the plan.
3. **Reinforce protein and resistance training before the last dose.**
4. **Agree a re-engagement threshold** in advance.
5. **Pregnancy planning** — use the medical director's discontinuation interval.
6. **Supply interruption:** per-patient prescriber decision, documented.
7. **Restarting after a gap:** lower dose, re-titrate. Never resume at the old dose.
8. **Discharge summary + letter to primary care.** Close the loop.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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